Statement of Deficiency Report

Department of Health
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Pearl Street 1201 S. Proctor St. Tacoma, WA. 98405 Agency Name and Address		Melissa Olsen Administrator
Investigation Inspection Type	Wednesday, October 7, 2020 Investigation Onsite Date	42124 Investigator Number
2020-12364 Case Number	BHA.FS.61067737 License Number	Behavioral Health BHA/RTF Agency Services Type

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the investigation.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction
WAC 246-341-0410 (1)(c)(2)(b) Agency administration—Administrator key responsibilities (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (c) Meeting all applicable rules, policies, and ethical standards (2) The administrator must: (b) Ensure administrative, personnel and clinical policies and procedures are adhered to and kept current to be in compliance with the rules in this chapter, as applicable	Based on interview, clinical record review and policy and procedure review, the facility failed to follow internal policy and procedures and appropriately intervene when residents were engaged in inappropriate sexual behaviors for 2 of 2 residents reviewed (Residents #1 and #2). Failure to protect residents from inappropriate sexual behaviors can retraumatize residents and lead to poor resident outcomes. Findings included:	

	1. Review of the policy titled, "Treatment Milieu-Permissible Touching", issued 12/87 and revised 07/20, shows the guidelines for peer to peer appropriate touching. "Resident to resident touching is allowed only with appropriate caution, supervision and direction of treatment staff... The following types of touch are prohibited: any unwanted touch...touch that is sexual in nature...staff will intervene when touch is inappropriate." 2. Review of the clinical record for Resident #2 shows the following: a. Review of the Master Assessment dated 02/04/20 shows "Short, long term goals" as, "[Resident #2] needs to work on aggressive behaviors, physically, sexually and verbally." b. Review of the Master Assessment dated 02/04/20 shows Resident #2 denied a history of sexual abuse, however "[Resident #2] had some difficulty recalling the past history and much of the information is taken from the accompanying record... The record indicates significant concern with sexualized behavior. They were obsessed with a [person] starting in 6th grade with stalking. They were observed by [a family member] to be masturbating in the living room, and was asking peers to pull down their pants." 3. An email exchange with Staff A on 10/28/20 at 1:11 PM, shows that Resident #1 and #2 exposed their genitals to one another in March of 2020 as they were exiting the bathrooms.	

4. Review of agency notes between staff members regarding Resident #1, dated 03/27/20 at 1:40 PM shows "Resident #1 and Resident #2 flirting a lot. Has not led to poor boundaries." A note dated 03/27/20 at 9:44 PM, shows that Resident #1 and #2 were sitting away from one another due to playing footsie.

5. Review of agency notes between staff members regarding Resident #1 dated 05/02/20 at 9:58 PM shows, "[Another resident] approached me [staff] saying that Resident #1 said they want to have sex with [another resident] and [Resident #2], believes they saw Resident #1 forming a list of things they wished to experience sexually with these residents which made both [other residents] feel uncomfortable."

6. Review of agency notes between staff members regarding Resident #1 dated 05/04/20 at 7:46 AM shows Staff A stating, "Thank you [staff]- we should be watching interactions and boundaries closely with this group. I am concerned about Resident #1's vulnerability and also want to make sure all residents feel safe."

7. Review of agency notes between staff members regarding Resident #1 dated 05/04/20 at 11:23 AM shows that a staff member expressed their concern to Resident #1 about "the bathroom incident", but Resident #1 felt what "Resident #2 did was a one-time thing and it wouldn't happen again."

8. Review of agency notes between staff members regarding Resident #2 dated 05/08/20 at 1:53 PM shows, "Make sure to watch boundaries with [Resident #1]. Conversations about a prom and getting upset and emotional with each other. Also sitting too close to each other. [Resident #1s] spot in the hall is getting moved to the side by the bathrooms so there is more distance between the two."

9. Review of agency notes between staff members regarding Resident #2 dated 05/09/20 at 1:26 PM shows, "I [staff] noticed their [Resident #1 and #2's] lack of boundaries yesterday- sitting close to each other and what sounded like an argument but I was not in the hallway long enough to hear the content of their conversation."

10. Review of agency notes between staff members regarding Resident #2 dated 05/21/20 at 6:48 PM shows, "I [staff] have noticed the relational dynamic between [Resident #2] and [Resident #1]. On Tuesday, [Resident #1] asked me if they could have [Resident #2's] sweatshirt to which I replied, 'no' and explained the policy regarding sharing clothes. I also asked them to physically distance as their knees were touching. They clearly need guidance and practice on appropriate boundaries".

11. Review of the note for Resident #1 and #2 dated 06/01/20 at 7:47 PM states the following: "During cinema therapy group [Resident #1] and [Resident #2] were attempting to play footsie. [Resident #1] attempted to continue to play

	footsie with [Resident #2] even after [Resident #2] disengaged from that. I [staff] had a one-on-one conversation with [Resident #1 and #2] (sic separately) after group. At first, they seemed confused why they would no longer be able to sit next to [Resident #1 and Resident #2] during cinema therapy group; however, when I directly called them out for playing footsie with [Resident #1 and #2], they immediately owned up and apologized. I told them that there will never be a shortage of [people] who think they are cute; however, this is a treatment facility and not the place to develop those kinds of relationships. They were accepting that they will no longer be able to sit next to each other in future cinema therapy groups." 12. Review of agency notes between staff members regarding Resident #2 dated 6/3/20 at 12:44 PM showed, "[Resident #2] disclosed to staff that they and [Resident #1] have touched each other in the private area and kissed while they were on the couch a couple weeks ago. [Resident #1] is down hallway B. We started room changes already just haven't completed them."	

Washington State Department of Health
PLAN OF CORRECTION

Investigation: October 7, 2020 Investigation Number: 42124 License Number: BHA.FS.61067737 Case Number: 2020-12364	Comprehensive Life Resources Pearl Youth Residence (PYR) Formerly Pearl Street Center

Deficiency Number and Rule Reference	Correction	Responsible Party	Implementation Date	Maintaining Correction
WAC 246-341-0410 (1)(c)(2)(b) Agency administration—Administrator key responsibilities (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (c) Meeting all applicable rules, policies, and ethical standards (2) The administrator must: (b) Ensure administrative, personnel and clinical policies and procedures are adhered to and kept current to be in	1.Since the DOH investigation, there has been a building location change. The new building has more physical space and the staff can be more strategic about which residents can be in the same unit. This new facility is now Pearl Youth Residence. 2. Physical Boundaries Observation Log: Currently, in response to the pandemic and resident struggles to maintain healthy boundaries, Pearl Youth Residence is operating as a no-touch milieu. In order to ensure this and that our Permissible Touch Policy is enforced, we will create a Physical Boundaries Observation Log which will be completed by milieu supervisors either in person or via cameras at least twice weekly. The log will include: name of observer, date, activity in progress/location, unit being observed, and notes for observations and interventions.	2. Log will be created by Program Director	2. Log will start 11/21/20	2. Program Director and Milieu Manager will review/monitor the use of the log on 12/4/20. It will be checked/reviewed again on 2/3/21 and again in April 2021, at which time we will make the determination if it needs to be continued.

compliance with the rules in this chapter, as applicable	3. Room Change Spreadsheet: Earlier this year we created a room change spreadsheet in order to better assess and track appropriateness of room changes for residents. It is a priority to be intentional and strategic about unit and room assignments in order to maintain safety, security, and supervision, as well as provide the most advantageous environment for treatment. The spreadsheet includes: resident name, date, diagnosis, 5ft no contact restrictions, behavioral intervention reports and incident reports, property destruction, current room number and proposed new room number.	3.Room Change Spreadsheet will be updated by Program Director	3. By 11/19/20 we will revise the spreadsheet to include specifically: behavioral concerns (such as aggression), sexually acting out behaviors (present and history), and victimization. Milieu Manager will review the spreadsheet to ensure it is being used consistently and properly.	3.Milieu Manager will monitor that the spreadsheet is being using regularly on 12/3/20 and 1/3/21 and report to Program Director The spreadsheet will be reviewed for effectiveness and a determination will be made if monitoring needs to continue.
	4. Education and communication: In addition to the observation log and room change spreadsheet, we will continue to communicate and provide additional training/reminders for staff regarding the Permissible Touch policy and additional protocols put in place to address boundaries and COVID precautions. Program Director will make the first post in Teams (communication hub for all staff) by 11/20/20 that will include the Permissible Touch policy and a reminder about the additional protocols	4. Program Director	4. 11/20/20	4. Leadership will provide further education and training to staff based on observations noted by the unit supervisors' entries in the Physical Boundaries Observation Log. .

	we are currently following (no-touch milieu, observation log, social distancing, etc.).			

STATE OF WASHINGTON

DEPARTMENT OF HEALTH

November 16, 2020

Pearl Street
1201 S. Proctor St.
Tacoma, WA. 98405

Re: Case Number: 2020-12364
License Number: BHA.FS.61067737
Acceptable Plan of Correction
Date of Survey: October 7, 2020

Dear Administrator:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the investigation recently conducted at your facility, the Department has determined that the POC is acceptable. You stated in your plan that you will implement corrective actions by the specified timeline. By this, the Department is accepting your Plan of Correction as your confirmation of compliance.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department will not conduct an unannounced follow-up compliance visit to verify that all deficiencies have been corrected. Any fees that may be enforced based on the results of the investigation will be implemented consistent with WAC 246-341-0365 and WAC 246-337-990

In addition, the Department reserves the right to pursue enforcement action for any repeat and/or uncorrected deficiencies based on applicable statute and rules.

Please feel free to contact me at the email address you have on file if you have questions regarding the investigation.

I appreciate your cooperation and time during the investigation.

Investigator 42124
Behavioral Health Investigator
Health Systems Quality Assurance Division
Washington State Department of Health